

MP-104: Attended In-Laboratory Polysomnography (Sleep Study)

Bellcourt Medical Policy | Version 1 | Status: ACTIVE | Effective for dates of service 2024-01-01 until superseded

Field	Value
Policy number	MP-104
Service code(s)	BHA-DX-9581
Applies to	All clients administered by Bellcourt unless the client's plan document or delegation addendum states otherwise
Owner	Clinical Policy Committee
Version	v1

1. Purpose and scope

This policy sets out the clinical criteria Bellcourt reviewers use to decide whether the service described in this policy (Attended In-Laboratory Polysomnography (Sleep Study)) is medically necessary. It is applied to prior authorization requests where the date of service falls within the effective period stated above. A client's plan document may exclude this service or impose additional requirements; plan provisions are checked first.

2. Definitions

Medically necessary: services that are appropriate and consistent with the diagnosis, in accordance with generally accepted standards of medical practice, and not primarily for the convenience of the member or provider.

3. Coverage criteria

The service is considered medically necessary when the criteria below are met, read together using the AND / OR connectors shown:

- 1 A home sleep apnea test (HSAT) was performed and was negative, inconclusive, or technically inadequate;
OR
- 2 The member has a significant comorbidity that makes HSAT unreliable (moderate-to-severe COPD, heart failure, neuromuscular disease); OR
- 3 A sleep disorder other than obstructive sleep apnea is suspected (narcolepsy, parasomnia, periodic limb movement disorder).

4. Not medically necessary / limitations

- Initial evaluation for suspected OSA in a member without the comorbidities above, when HSAT has not been attempted.
- Repeat diagnostic testing after a positive HSAT.

5. Documentation requirements

Sleep history and Epworth score; HSAT report if performed; documentation of relevant comorbidities.

If required documentation is missing, the request must be pended and the provider contacted for information. It must not be denied for lack of documentation until the information request has gone unanswered within the regulatory timeframe.

6. Version history

Version	Effective	Summary
v1	2024-01-01	Initial version.